

UP-22 SEXUAL ASSAULT

Universal Patient Care | EMT - AEMT - Paramedic

Core principle: Treat immediate medical needs, protect safety and privacy, obtain consent throughout care, preserve potential evidence without delaying treatment, and respect a competent adult's reporting decision unless a mandatory child, vulnerable-adult, or qualifying-injury requirement applies.

QUICK FLOW

1. SAFE + PRIVATE? Separate from suspected assailant; request law enforcement for immediate danger.	2. TREAT LIFE THREATS XABCDE, bleeding, airway, shock, pain, pregnancy or strangulation risk.	3. MINOR / VULNERABLE? Yes: protect patient and initiate required DCS or APS reporting.
4. INJURY REPORT? Life-threatening injury, strangulation, knife, firearm, or deadly weapon.	5. ADULT PREFERENCE Explain options; respect law-enforcement choice unless an exception applies.	6. PRESERVE + TRANSPORT Minimal handling; appropriate medical/forensic destination; private handoff.

TENNESSEE REPORTING DECISION

PATIENT / FINDING	FIELD ACTION
Child under 18	Suspected sexual abuse is a mandatory child-abuse report. Initiate the approved DCS pathway; hospital notification alone does not replace the reporter's duty.
Vulnerable adult	Suspected sexual abuse requires the approved Adult Protective Services pathway. Assess capacity and vulnerability without assuming age or disability alone removes autonomy.
Competent, non-vulnerable adult	The sexual assault is not automatically reported to law enforcement when the patient objects to release of identifying information. Explain options and respect the patient's choice.
Qualifying injury	Report life-threatening injury or injury inflicted by strangulation, a knife, firearm, or other deadly weapon. This is an injury report; the patient is not required to speak with law enforcement.

PROVIDER-LEVEL ACTIONS

EMT	Treat immediate threats; obtain only the history needed for care and safety; preserve privacy and evidence; avoid a forensic interview or unnecessary genital exam. If transporting, privately notify ED staff of the suspected cause of injury.
AEMT	All EMT actions plus authorized vascular access, fluids, analgesia, antiemetic therapy, and monitoring. Reassess for occult shock, strangulation, intoxication, pregnancy-related concern, and qualifying injuries.
PARAMEDIC	All prior actions; lead capacity, destination, Medical Control, refusal, and reporting decisions. Distinguish patient-authorized law-enforcement involvement from mandatory protective-services or injury reporting.

ASSESSMENT

- Assess scene danger, suicidal thoughts, significant bleeding, head injury, strangulation, intoxication or overdose, pregnancy-related emergency, and other time-critical illness or injury.
- Ask only what is needed for treatment, safety, destination, and evidence preservation: event timing; pain or injury; strangulation; bleeding; loss of consciousness; weapon use; possible drugging; pregnancy possibility; and bathing, changing clothes, eating, drinking, urination, or defecation since the event.
- Use the patient's exact words. Do not demand a chronological account, challenge inconsistencies, ask why, or repeat questions already answered unless clinically necessary.
- Perform only the exposure and examination needed for emergency care. Obtain consent before each step. A normal field examination does not exclude sexual assault or significant injury.

PATIENT-CENTERED CARE

- Use validating language: "I am sorry this happened," "This is not your fault," and "You are in control of what happens next."
- Explain each action before touching or exposing the patient. Allow the patient to decline any nonessential assessment or procedure.
- Do not require a police report or law-enforcement cooperation as a condition of medical care or transport.
- Request law enforcement for immediate danger, a required report, or when the patient requests assistance. Do not confront the suspected assailant or investigate the crime.

EVIDENCE PRESERVATION

- Advise that bathing, changing clothes, eating, drinking, smoking, brushing teeth, urinating, defecating, or cleaning may affect evidence, but never withhold necessary care, comfort, food, drink, toileting, or treatment.
- Avoid unnecessary handling. If clothing must be removed for care, keep separate items separate, use clean dry paper bags when available, and document transfer under agency policy. Do not place damp biological evidence in plastic.
- Do not collect swabs or independently assemble a sexual-assault evidence kit in the field. Medical stabilization and patient well-being take priority over evidence.

TRANSPORT, HANDOFF, AND NONTRANSPORT

- Transport to an appropriate facility with sexual-assault forensic capability when available and consistent with medical needs and patient choice. Limit open-radio details to necessary clinical information.
- When transporting, privately notify the receiving physician or ED staff of the suspected cause of injury and provide objective findings, treatment, reporting criteria, and evidence handled.
- If the patient is not transported, follow the current Tennessee first-responder requirement and agency procedure for reporting the result of the call to the 911 center. Use Medical Control for high-risk refusal or questionable capacity.
- An adult may receive a forensic medical examination without immediately filing a police report; Tennessee permits a non-reporting kit to be retained through the hold-kit process.

HIGH-RISK WARNING: STRANGULATION

Visible injury may be absent. Voice change, dysphagia, dyspnea, neck pain or tenderness, petechiae, loss of consciousness, incontinence, seizure, confusion, focal deficit, or a memory gap may signal serious airway or vascular injury. Transport for urgent evaluation and complete the required injury report.

DO NOT

- Do not make a blanket law-enforcement report for every competent adult sexual-assault patient.
- Do not promise complete confidentiality; explain child, vulnerable-adult, and mandatory injury-reporting limits.
- Do not conduct a forensic interview, pressure the patient to report, or perform an unnecessary genital examination.
- Do not delay lifesaving care, necessary toileting, comfort, or transport solely to preserve evidence.

DOCUMENTATION

- Record exact patient statements in quotation marks, objective findings, capacity, consent or refusal for care, patient preference regarding law enforcement, reporting criteria considered, and all required reports or notifications.
- Document each potential evidence item handled, packaging, chain of transfer, destination decision, receiving clinician, and private handoff.

DRAFT FOR REVIEW. Final Claiborne reporting contacts, forensic-capable destinations, advocacy resources, refusal process, evidence procedures, and documentation requirements must be approved before clinical release.

References: T.C.A. 38-1-101; Tennessee DCS mandated-reporter guidance; Tennessee Adult Protective Services; Tennessee OCJP Best Practice Guidelines for Sexual Assault Response; Tennessee Bureau of Investigation Sexual Assault Kit and Hold Kit guidance; Claiborne County EMS local policies.